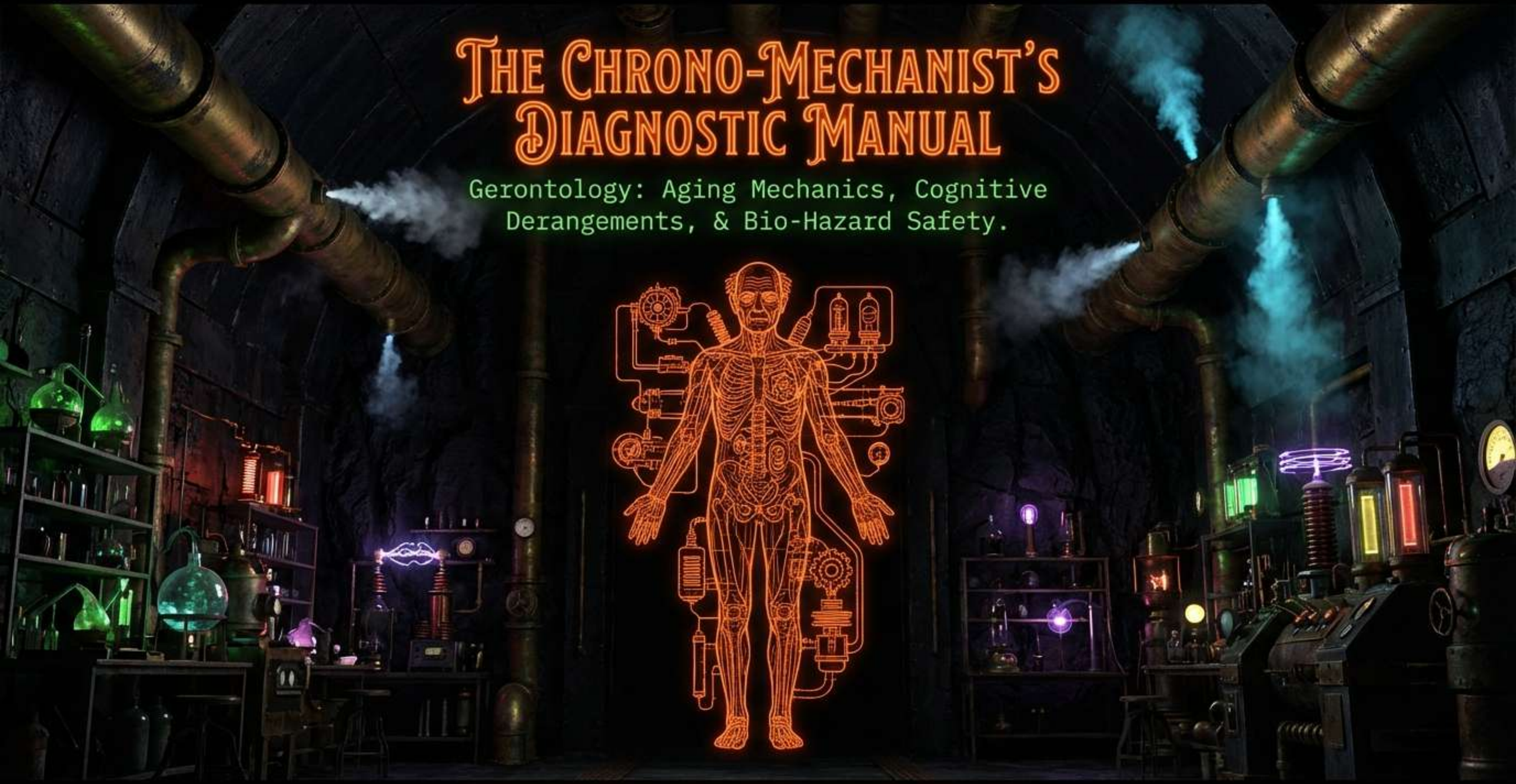
The Chrono-Mechanist's Diagnostic Manual

Gerontology: Aging Mechanics, Cognitive Derangements, & Bio-Hazard Safety.



Protocol based on Mark Klimek Clinical Directives.

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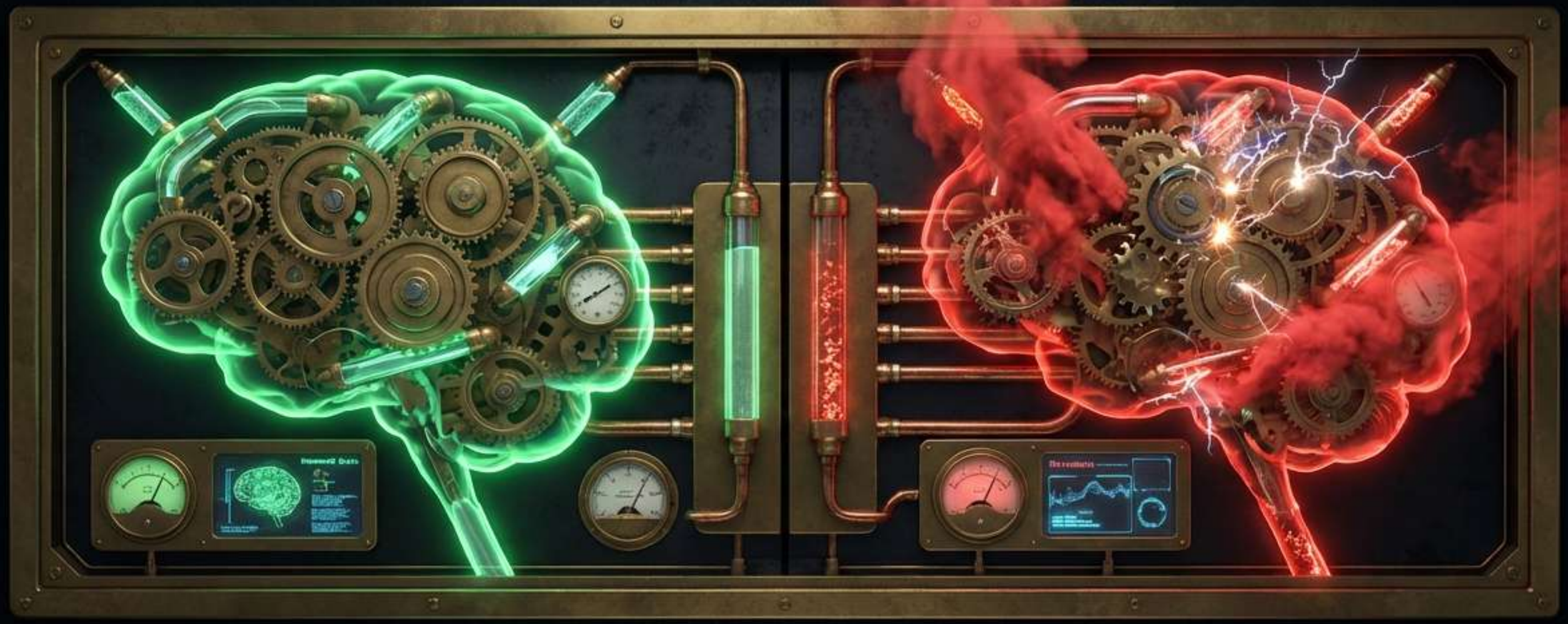
NON-PSYCHOTIC

PSYCHOTIC



Insight Intact.
Reality-Based.
Action: Therapeutic
Action: Therapeutic Communication.

No Insight.
Not Reality-Based.
Blames Others.
Action: Protocol Override Required.

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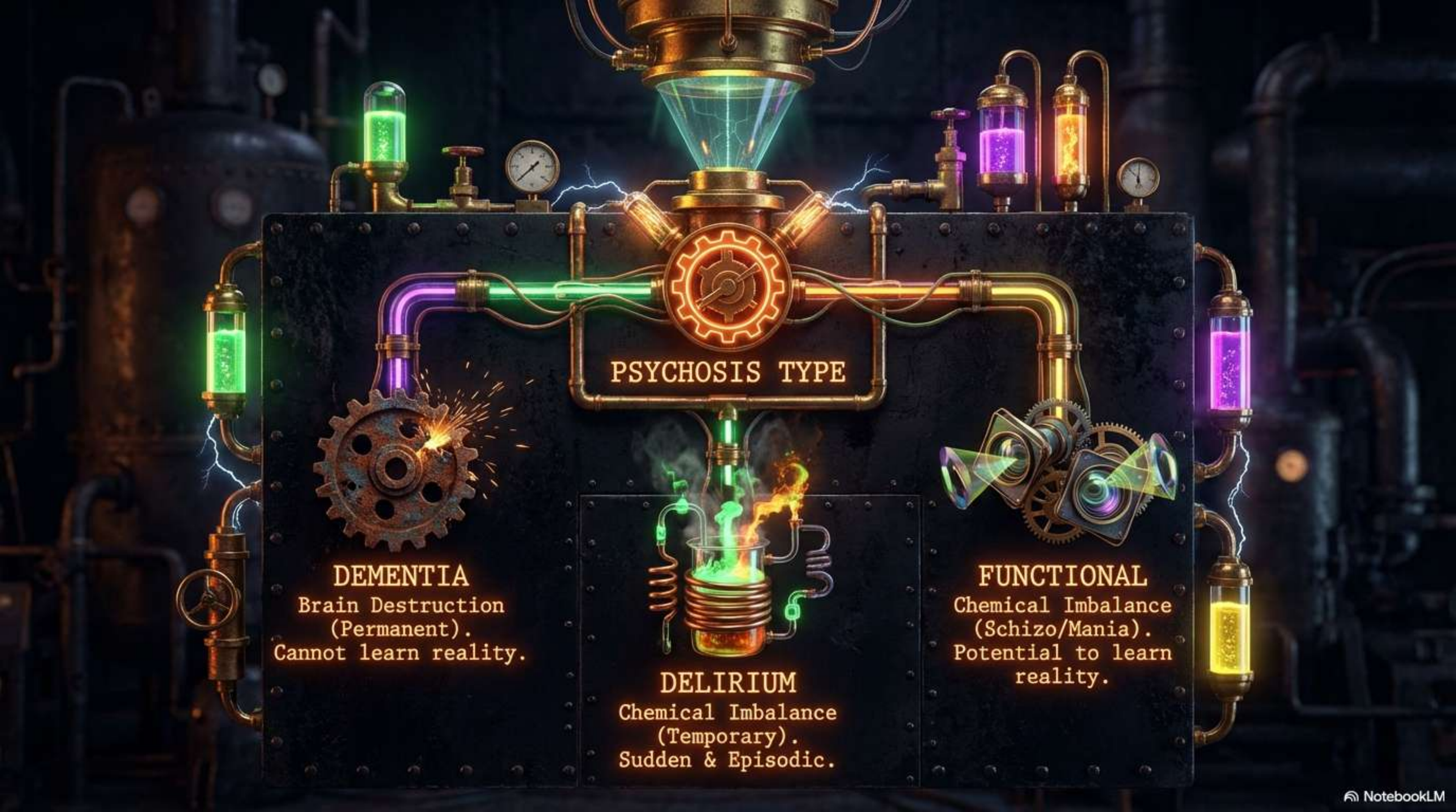
PSYCHOSIS TYPE
DEMENTIA
Brain Destruction (Permanent).
Cannot learn reality.
DELIRIUM
Chemical Imbalance (Temporary).
Sudden & Episodic.
FUNCTIONAL
Chemical Imbalance (Schizo/Mania).
Potential to learn reality.

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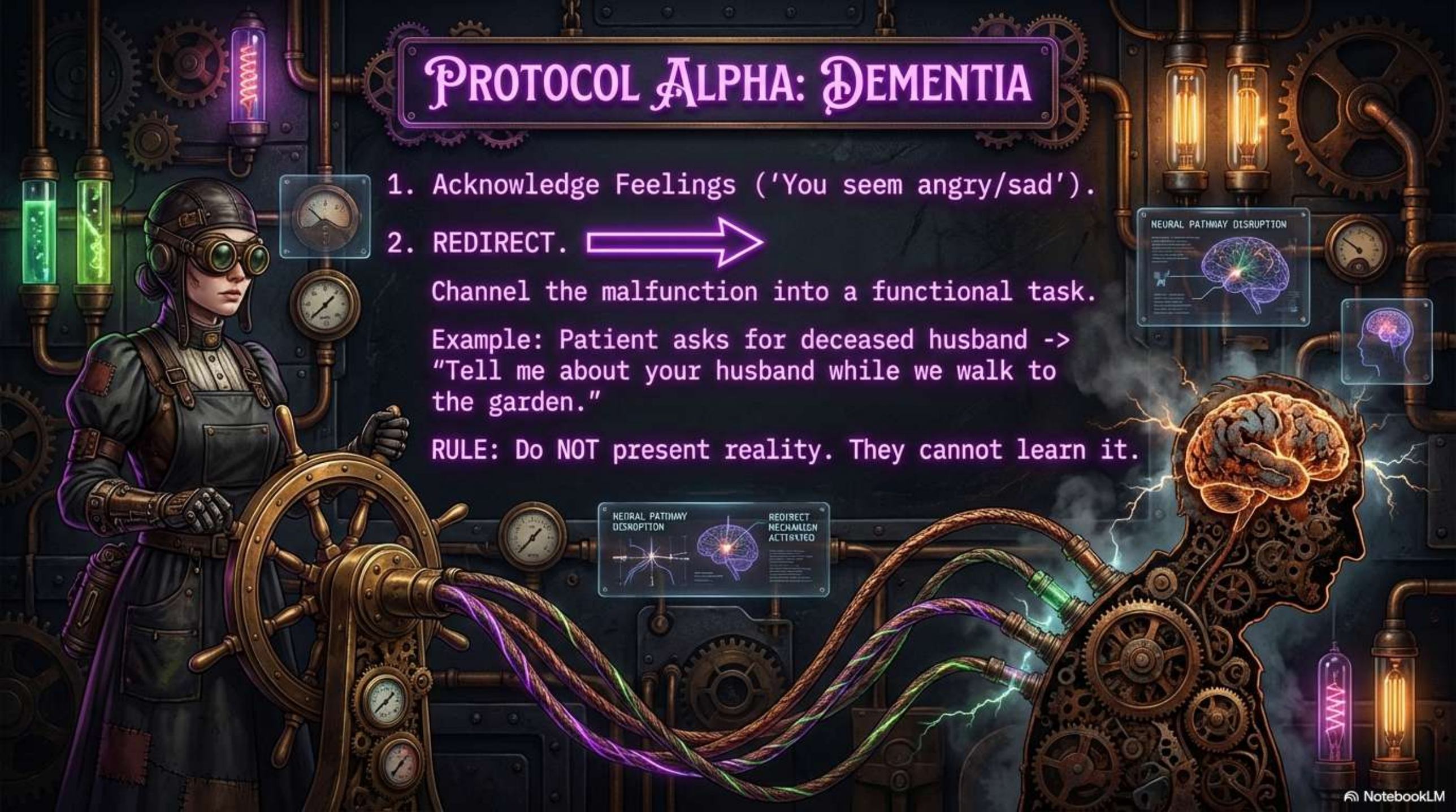
Protocol Alpha: Dementia

1. Acknowledge Feelings ('You seem angry/sad').
2. REDIRECT.

   Channel the malfunction into a functional task.

   Example: Patient asks for deceased husband -> "Tell me about your husband while we walk to the garden."

   RULE: Do NOT present reality. They cannot learn it.



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Reality Orientation vs. Presenting Reality

Reality Orientation

Orient to Person, Place, Time.

ALWAYS APPROPRIATE for Dementia.

Presenting Reality

Correcting the delusion (e.g., "Your husband is dead").

NEVER DO THIS. Causes frustration.

Keep the machine anchored to the 'Now,' but do not fight the Phantom.

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Protocol Beta: Delirium

CAUSE: Chemical Imbalance (UTI, Thyroid, Electrolytes). Sudden & Dramatic.

ACTION (Neon Purple):

1. Acknowledge Feelings.
2. REASSURE.
   - "You are safe."
   - "This is temporary and will go away when you get better."

WARNING (Neon Red):

DO NOT REDIRECT. You cannot redirect a chemical imbalance.

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WERNICKE-KORSAKOFF SYNDROME

Pathology: Psychosis induced by Vitamin B1 (Thiamine) deficiency.

Primary Symptom: CONFABULATION (Making up stories to fill memory gaps).

The 3 Characteristics:

1. **PREVENTABLE:** Take B1.
2. **ARRESTABLE:** Stop progression (Take B1).
3. **IRREVERSIBLE:** 70% of cases kill brain cells.

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THE HONEST LIAR: WERNICKE'S PROTOCOL

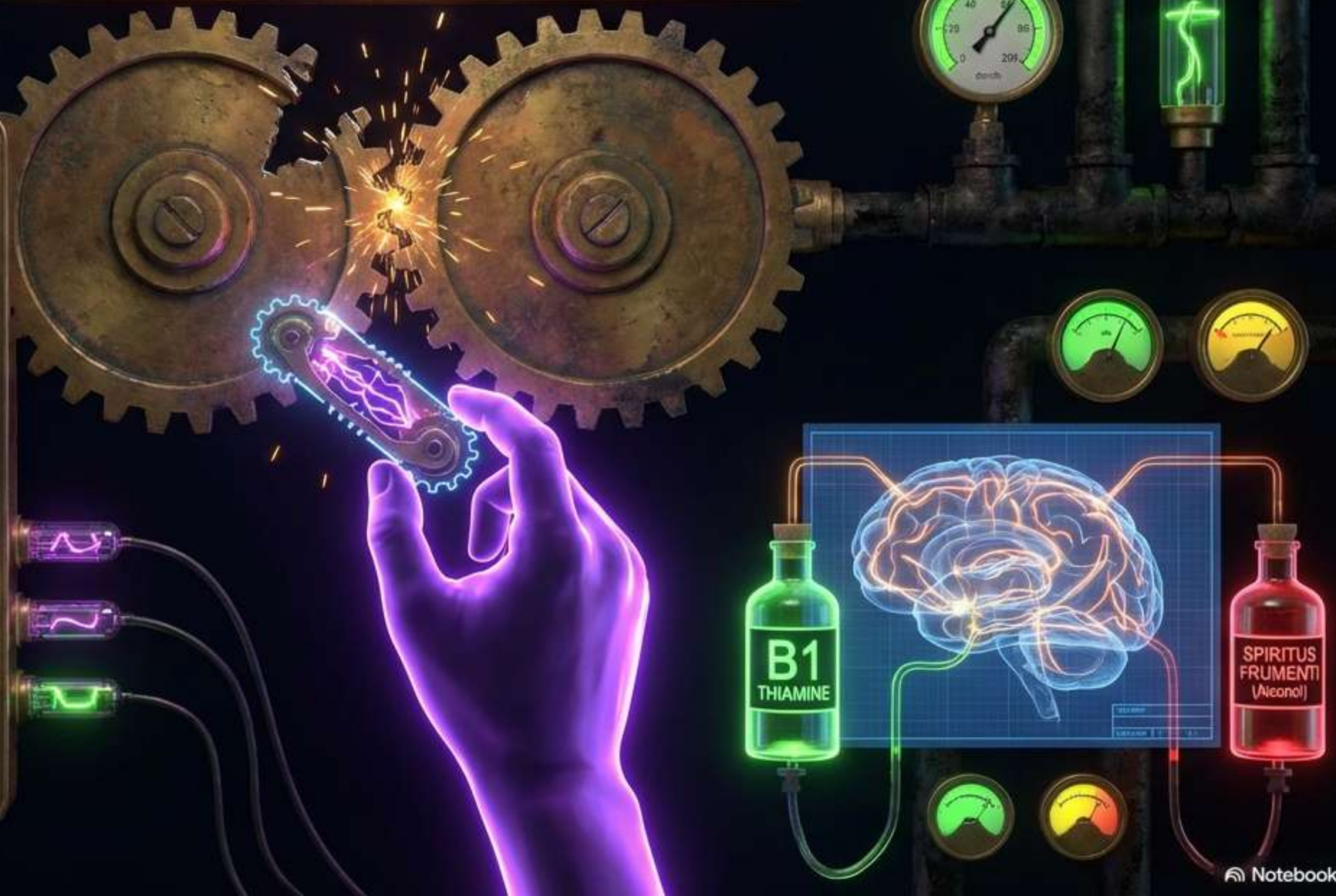
INTERVENTION:
Do not confront the lie.
They are confabulating
because reality is lost.

ACTION: REDIRECT.

Scenario: Patient claims they
are meeting the President
for breakfast.

Response: "Let's go watch
the news to see what the
President is doing today."
(Redirect to a concrete task).

Dietary Directive: Reduce
Alcohol, Increase B1.



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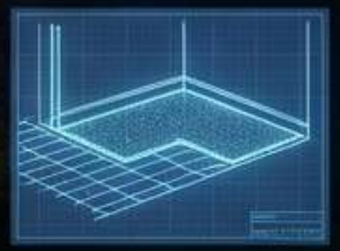
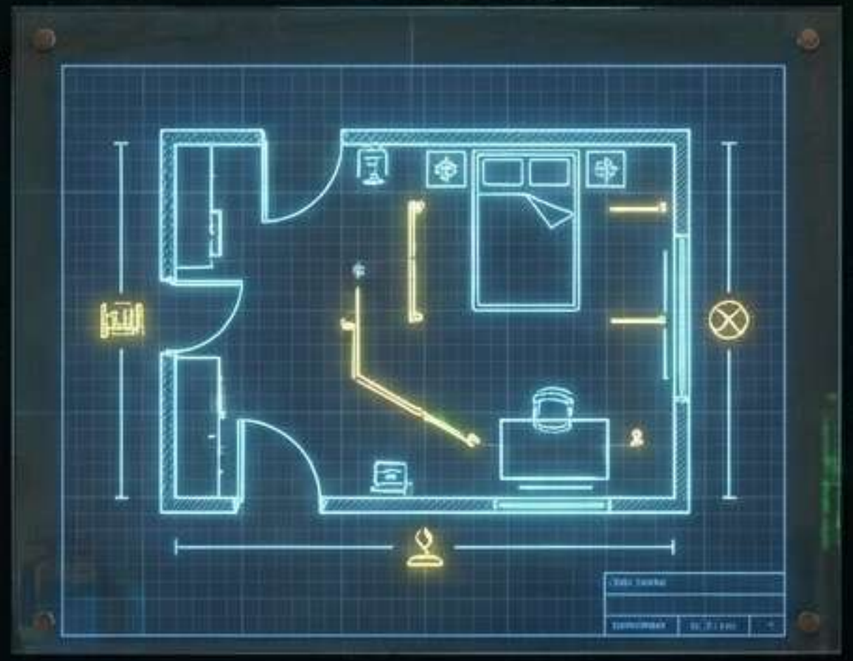
HABITAT ENGINEERING: ALZHEIMER'S

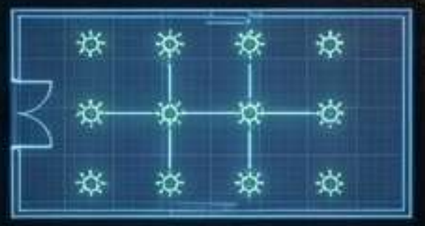
- SIGNAGE: Clear, simple symbols on doors (Restroom, Bedroom). Processor cannot map complex routes.

- FLOORING: Tacked down carpets only. NO throw rugs.





- LIGHTING: Consistent, low-shadow illumination.



- CLUTTER: Minimal. Clear pathways.

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CRITICAL WARNING: THE PHANTOM

NO MIRRORS

or complex artwork.

Mechanism: Patient does not recognize their own reflection. They perceive an INTRUDER.

Result: Severe agitation, fear, and hallucinations.

Protocol: Cover or remove reflective surfaces in the Alzheimer's unit.

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The Silent Alarm: Atypical Presentation

The Broken Sensor:
Elderly patients often **DO NOT** fever or chill with Pneumonia or UTI.

The True Signal:
Sudden onset of **CONFUSION**, Lethargy, Weakness, Anorexia.

DIAGNOSTIC RULE: If a geriatric patient suddenly acts "crazy" or confused, suspect **OCCULT UTI** or **Hypoxia** first.

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Structural Integrity: Fall Prevention

The Hazard:
Hip fractures carry high mortality in the elderly.

CAUTION: WET FLOOR

PROTOCOLS:

1. Gait Belts: Mandatory for transfers.
2. Environment: Remove scatter rugs. Install grab bars.
3. Sensory: Night lights in bathrooms (Sensory deprivation causes falls).

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MECHANICAL EXOSKELETON AIDS

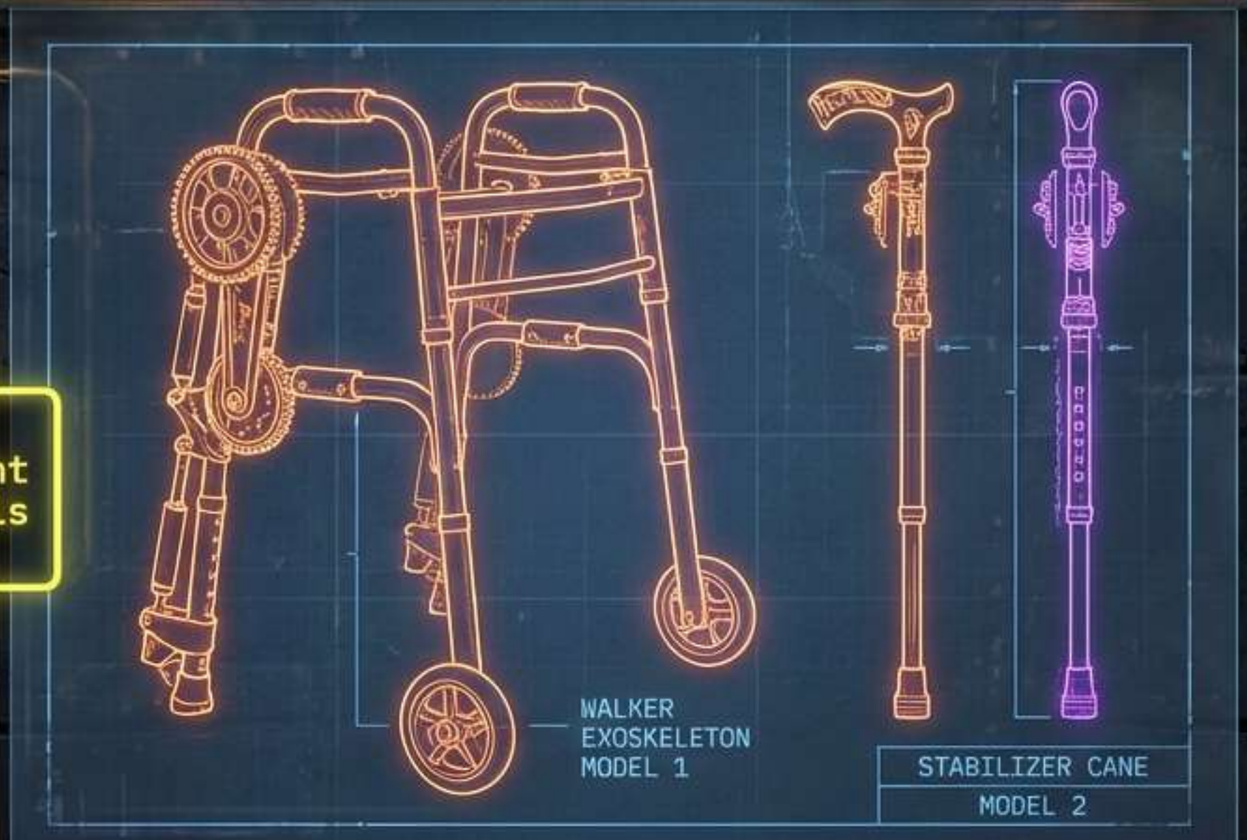
THE WALKER PROTOCOL:

1. Pick it up.
2. Set it down.
3. Walk to it.

CAUTION: Do NOT tie belongings to the front (Tip Hazard). No wheels unless prescribed.

RISING FROM CHAIR:

Hold chair arms, stand up, THEN grab walker. Never pull up on the walker.



THE CANE PROTOCOL:

Hold on the STRONG side.

Advance cane with the WEAK leg.

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PHARMACOLOGY: THE HALF-DOSE RULE

Mechanism: Age decreases liver metabolism and renal excretion.

Risk: Standard adult doses = Overdose/Toxicity in the elderly.

PROTOCOL: Start low, go slow. Usually 1/2 the adult dose.

Tranquilizers: Avoid if possible (fall risk). If necessary, use non-sedating options.

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ANTIPSYCHOTIC TOXICITY

EPS GAUGE

EPS (Extrapyramidal Symptoms).
Tremors, rigidity, shuffling.
Side Effect. Not life-threatening.
Treat with Cogentin.

NMS GAUGE

NMS (Neuroleptic Malignant Syndrome).
MEDICAL EMERGENCY.

- Signal: HIGH FEVER (102°F – 105°F+).
- Action: HOLD MED. MEASURE TEMP. CALL DOCTOR.
- Note: Elderly are at highest risk.

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THE SHADOW OPERATOR: ABUSE PROTOCOLS

The Dynamic: Abuser is often a family member or caregiver.
The Defense: Victim will often DENY the abuse due to fear or dependency.

THE NURSE'S CODE

1. Assess: Look for unexplained bruises, malnutrition, withdrawal.
2. Action: MANDATORY REPORTING.
3. Priority: Remove patient from immediate danger zone to assess safely.

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OPERATIONAL SUMMARY

- [ ] DIAGNOSIS: Distinguish Rusted Gears (Dementia) from Chemical Glitches (Delirium).
- [ ] ACTION: Redirect the permanent; Reassure the temporary.
- [ ] SENSORS: Watch for Confusion, not Fever.
- [ ] SAFETY: Clear floors, half-doses, no mirrors.
- [ ] PRIME DIRECTIVE: Maintain dignity and structural integrity.

STATUS: SHIFT COMPLETE.

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